

Case 9: Sweaty Boots (Tinea) Case Guide

1. DESCRIPTION OF THE STATION

In this station, students will roleplay as community pharmacists. A simulated patient (SP) will present with a consumer query.

The students' tasks are to:

- Perform a comprehensive history-taking to any identify medical/medication related problems, and then
- Communicate their recommendations (pharmacological and/or nonpharmacological) and follow up to the simulated patient.

Process

- This station will have **only an examiner** who will also roleplay as the **patient/carer**.
- Upon entering the room, students should state their name and ID number to the recording.
- The case stem will be available on the table for the student to read.
- Students should initiate the interaction.
- Students will conduct a thorough history-taking, and the SP will respond according to the script.
- Students are allowed to use their references throughout the whole OSCE session.
- The total time for this station is **5 minutes** (from when the student introduces themselves to when they conclude the interaction).
- There will be a **3-minute buffer** between each student.

[Please refer to the Monash University Malaysia Good OSCE Practice for Examiners and SPs for detailed explanations on the standards of role playing and examining in BPharm OSCE]

2. EXAMINER TO SAY TO STUDENT

"Please state your name and student number for the recording. Your case stem is available for you on the table, and you may begin when you are ready."

3. STUDENT READS THIS (CASE STEM)

You are the community pharmacist working today.

A customer presents to your pharmacy and approaches you for assistance.

Respond as you would in practice.

4. DESCRIPTION OF PATIENT *[The role you play]***Name:** Any (say your real name)**Gender:** Either (say your actual gender)**Age:** Any (say your actual age)**5. PATIENT COMMUNICATION****NON-VERBAL:** FriendlyOpening statement: *'My feet are driving me crazy. Can you give me something to fix it quickly?'*

Please follow the script below to give any information if asked.

If the answer is not on the script, tell the student: *"I do not have the information"*If the student refers you to a doctor, say *"I'd like something to help with the symptoms now."*If student offers a product but doesn't counsel you, ask *"How do I use the medicine?"*

Who is the patient?	Yourself . Name: Say your real name . Registration number: URN 234567 . Address: 2 Jalan Sultan, 50000 Kuala Lumpur . DOB: Say your real DOB . Age: Say your real age . Gender: Say your gender . Weight: Say your actual weight . Height: Say your actual height
Pregnant/breastfeeding [if female]	No
Allergies (nature of reaction)	Peanuts (immediate anaphylaxis)
Current medical conditions	None
Current medications	None
OTC, herbal, complementary medicines, or other medicines in the form of patches, injections, nasal sprays, eye drops, creams or ointment	None
Recent changes to current medications	None
Symptoms	<u>Today's chief complaint</u> Flaking and itchiness between the toes <u>Divulge the following if prompted for more symptoms or why you need a product</u> Flaking and itchiness between the toes. Skin is red with cracking and peeling between the two little toes on each foot. The itching started after a long hot day working on the farm. <u>Reply "No" if prompted for any of the following referral symptoms</u> Weeping, odour, purulent ulceration, pus, heat, swelling or even fever

Timeline/ duration of symptoms	• 3 weeks ago - Been a bit itchy around the toes • Last 3 days - Itch has worsened and noticed cracking, redness and peeling skin between the toes
Do you know what caused the symptoms?	<i>"I noticed the itching started after a long hot day working on the farm."</i>
When was the last time you had similar symptoms?	Never had these symptoms before
Have you tried anything for the symptoms?	Haven't tried anything for symptoms yet
Preferred dosage form/ formulation	Cream
Dietary habits	Healthy diet
Exercise	Regular exercise (jogs alternate days)
Smoking	Never
Alcohol	Only has a drink on special occasions
Social history	Lives at home with partner. Farmers, works on his farm wearing boots most days

6. PROPS: None

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Analytical Checklist

GATHERING INFORMATION/ INTRODUCTION	YES	NO
1. Elicits that area around the patient's toes is itchy AND other symptoms of tinea (there is cracking, redness and peeling of skin between toes) (i.e. when asked to describe symptoms)		
2. Elicits that symptoms started three weeks ago after working on the farm wearing boots all day (i.e. when asked when it started)		
3. Elicits the patient has not had the symptoms before AND has not tried anything for symptoms yet AND has no signs of infection (no pus, swelling etc.)		
OPTION/MANAGEMENT STRATEGIES (INCLUDING PATIENT EDUCATION)	YES	NO
4. [CRITICAL CRITERIA] Advises patient has tinea OR fungal infection on the skin OR cause of itchiness due to wearing boots whole day (hot and humid)		
5. [CRITICAL CRITERIA] Recommends any of ONE the following topical therapy: Terbinafine, OR Miconazole/ clotrimazole/ econazole/ bifonazole/ ketoconazole, OR Combination hydrocortisone/antifungal		
6. Counsels on how to use chosen product in Point 5: - Terbinafine: Apply a thin layer once daily for 1 week - Miconazole: Apply a thin layer twice daily; continue for 2 weeks after symptoms resolve - Clotrimazole/Econazole: Apply a thin layer 2 or 3 times a day; continue for 2 weeks after symptoms resolve - Bifonazole: Apply a thin layer once daily; continue for 2 weeks after symptoms resolve - Ketoconazole: Apply a thin layer 1 or 2 times daily; continue for 2 weeks after symptoms resolve - Combination hydrocortisone/antifungal (available combinations are clotrimazole + hydrocortisone OR miconazole + hydrocortisone): Apply a thin layer 2 times a day for one week then continue with antifungal only		
7. Recommends at least ONE of the following future preventive action: maintain good foot hygiene; keep feet clean and dry especially after working at the farm; avoid sharing towels/footwear; use antifungal (or non-medicated) powder in shoes to reduce moisture; wear open-toed footwear; use cotton (non-synthetic) socks; wear shoes that have breathable material (such as leather); place shoes in the sun to dry if it get damp; discard old shoes that may have a high density of fungal spores		
MONITORING/FOLLOW-UP	YES	NO
8. Advises the patient to see their doctor /pharmacist if their symptoms do not improve or worsens		
Cut Score: 4/8		

References:

- APF26 > Sections: Treatment guidelines for pharmacists > Tinea
 - Link: <https://apf.psa.org.au/>, Username: vinod.kumar1@monash.edu, Password: MonashU24!
- AMH > Therapeutics: Tinea
 - Link: <https://amhonline-amh-net-au.ap1.proxy.openathens.net/>, use your Monash log in details
- MIMS Australia (use this to check product monograph and Poison Standard schedules)
 - Link: <https://www.mimsonline.com.au/Login/Login.aspx>, use your Monash log in details